

NEW

2026

Ethiopian Health Profession Licensing Examination(EHPLE)

INFORMATION BOOKLET

OPTOMETRY



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MINISTRY OF HEALTH - ETHIOPIA

የዜጎች ጤና ለሃገር ብልጽግና!
HEALTHIER CITIZENS FOR PROSPEROUS NATION!



AAU-Testing Center

Message From the State Minister, Ministry of Health -Ethiopia



H.E Mrs. Frehiwot Abebe

Improving healthcare quality is a global priority for sustainable development, with high quality healthcare being a key component of universal health coverage. One strategy to maintain health care standards is through provision of health professionals' competency assessment. Consequently, in 2019, the Ministry of Health-Ethiopia, initiated the Ethiopian Health Profession Licensing Examination (EHPLE) for undergraduates in seven health disciplines, which has since expanded to include 17 health disciplines.

The main goal of this competency assessment is to identify health professionals with minimal competencies necessary to perform their duties safely and competently, thus enhancing the quality of health care services. This initiative is overseen by a dedicated Health and Health Related Institutions and Professionals' Regulatory Lead Executive Office (LEO), comprising four desks, which plays a pivotal role in strengthening the system and enabling the LEO to conduct the competency exam more extensively and with improved organization and quality.

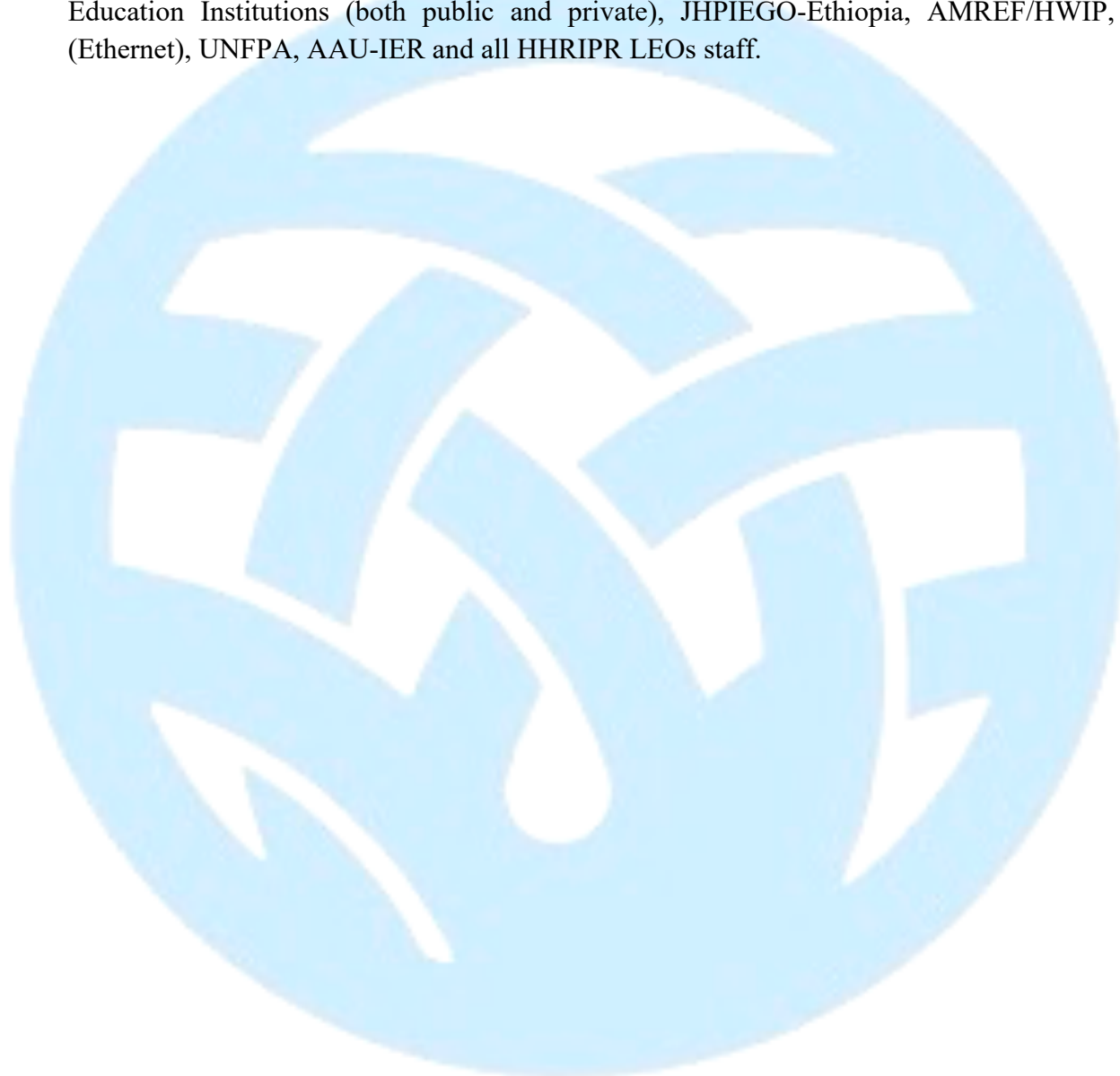
It is important to note that this competency assessment differs significantly from traditional academic or employment examinations. Hence, this information booklet has been created to address the informational needs of both examinees and teaching faculty regarding the EHPLE. Additionally, it aims to facilitate the assessment process, while promoting transparency and ensuring the sustainability of the program.

The preparation of this booklet involved the collaboration of esteemed experts from various higher education institutions, AAU-IER, the Ministry of Health, JHPIEGO-Ethiopia, Amref, Health Professionals 'Associations, and the Ministry of Education. Their invaluable contributions are acknowledged with sincere gratitude, alongside appreciation for the Ministry of Health staff for their unwavering commitment and hard work throughout the project.

Acknowledgements

This Information Booklet for EHPLE is a contribution from several educators, researchers, students and concerned individuals with a genuine interest to propel Ethiopia's medical and health sciences education forward.

The Ministry of Health is grateful for the contribution of many individuals and institutions in realizing this endeavor. Among these are Professional Associations, Student Association, Higher Education Institutions (both public and private), JHPIEGO-Ethiopia, AMREF/HWIP, MoE (Ethernet), UNFPA, AAU-IER and all HHRIPR LEOs staff.



Acronyms and Abbreviations

EHPLE	Ethiopian Health Profession Licensing Examination
ETA	Educational and Training Authority
HEIs	Higher Education Institutions
HHrIPR-LEO	Health and Health-related Institutions and Professionals Regulatory Lead Executive Office
HSTP-II	Health Sector Transformational Plan-II
MCQ	Multiple Choice Question
MOH	Ministry of Health
WHO	World Health Organization

Purpose of the Information Booklet

The EHPL E Information Booklet serves as a comprehensive guide for those individuals seeking information about the exam. It typically outlines basic information for candidate registration, exam development and administration processes and procedures, result notification, and the licensing process. It also includes information on the exam framework, i.e., the exam domain, sub-domain, content, process, and task, with sample exam items specific to each profession.

The publication of this Booklet is crucial for the following reasons:

- **Clarity and guidance:** It provides clear information about the exam by ensuring candidates understand the necessary information to prepare them.
- **Accessibility:** It serves as a readily accessible resource for individuals pursuing to take the exam, consolidating essential information in one document and facilitating easy access to necessary details. It also helps other stakeholders who might be interested in such resources.
- **Transparency:** It promotes transparency in the examination process and fosters trust among stakeholders about the exam.

In summary, the publication of this Booklet is essential for creating a transparent, standardized, and accessible framework that guides candidates through the EHPL E process.

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Definition of terms

- **Domain:** a broad category or area of knowledge or skills of a profession
- **Sub-domain:** a subset of a broader domain that focuses on knowledge or skills related to the overarching domain
- **Content:** a more specific subcategory, which is a breakdown of the sub-domain
- **Task:** the responsibility, knowledge, skill, and attitude of a junior undergraduate professional in an actual work environment
- **Process:** a systematic sequence of steps or actions designed to achieve a specific outcome
- **Learning outcome:** a clear and measurable statement that describes what the examinee is expected to know or be able to do
- **Relative emphasis:** the proportional importance or weight assigned to different content areas or categories within the assessment
- **Item:** a particular multiple-choice question
- **Item developer:** a subject matter expert responsible for writing test items or questions that make up the examination
- **Item reviewer:** a subject matter expert responsible for reviewing and refining the test items or questions that make up the examination
- **Standard setting:** a process of determining a cut-off point or passing score for an exam
- **Item difficulty index:** a statistical measure that indicates the proportion of examinees who answered a particular test item correctly
- **Discrimination index:** a statistical measure that evaluates how well a particular test item differentiates between high-performing and low-performing examinees
- **Admission paper:** a printout paper generated by the system after completing registration that contains the examinee's photo, QR code, and necessary information

1. Introduction

1.1. Background

Competency assessment is one of the strategies for controlling the standard of healthcare services provided in healthcare facilities. The World Health Organization (WHO) recommends all healthcare professionals to have necessary competencies. In Ethiopia, the Health Sector Transformational Plan-II (HSTP-II) states competency assessment of all graduates before joining the health workforce as one of the strategic initiatives.

The Ministry of Health (MOH) launched the Ethiopian Health Profession Licensing Examination (EHPLE) for undergraduates in 2019. The Health and Health-related Institutions and Professionals Regulatory Lead Executive Office (HHrIPR-LEO) of the Ministry of Health is given a mission to implement the Ministry's objective to achieve competency-related goals. It has the responsibility to ensure that the EHPLE meets technical, professional, and legal standards, and to protect the health, safety, and welfare of the public by assessing candidates' abilities to practice competently.

Currently, the exam is given for 17 health professions (Medicine, Nursing, Public Health, Pharmacy, Medical Laboratory Science, Anesthesia, Midwifery, Dental Medicine, Medical Radiology Technology, Environmental Health, Psychiatric Nursing, Pediatric and Child Health Nursing, Emergency and Critical Care Nursing, Human Nutrition, Surgical Nursing, Optometry, and Physiotherapy). Since its introduction until December 2025, a total of 204,811 examinees took the exam in 23 rounds.

1.2. The Rationale of EHPLE

One of the critical functions of the MoH is to guarantee the efficiency, quality, and equity of healthcare delivery and to protect the public from any undesirable consequences in healthcare delivery practices. As professionals' competence is a significant determinant of the quality of health, evaluation of health professionals' competence has now been given due attention. The licensing examination for health professionals serves as a crucial step to ensure that individuals entering the field meet specific competency standards. The sole aim of the competency assessment is to safeguard public health by verifying that health professionals have the minimal basic knowledge, attitude, and skill required to provide safe and effective care.

Licensing exams act as a preventive measure, ensuring that only competent professionals join the health workforce, which, in turn, contributes to reducing the occurrence of medical errors and enhancing overall patient safety. By setting standards through examinations, regulatory bodies strive to minimize the risk of medical errors caused by incompetence.

2. Key processes of EHPLE

EHPLE involves several key processes to ensure the quality and reliability of the examination.

2.1. Registration of candidates

EHPLE has a mandatory online registration system for both new and repeat candidates, which can be found at <http://hple.moh.gov.et>

Please note these important notes during registration.

New Test Takers:



- The list of eligible candidates from governmental and private Higher Education Institutions (HEIs) will be uploaded to MoH's online registration system by the registrar office of their respective HEIs.
- Candidates who completed their education abroad must bring the following documents in person to MoH in order for their names to be uploaded to the online registration system:
 - ✓ Equivalence document from ETA
 - ✓ Externship attachment completion letter after completing the externship assigned by the regulatory body
- Once the name of the candidate is uploaded to the system and registration has opened for the current exam round, the candidate must register at <http://hple.moh.gov.et> by uploading the necessary documents listed below.
 - ✓ a scanned original or temporary degree
 - ✓ a scanned national ID

Repeat Test Takers:



- Since the information about re-exam candidates already exists in the system, the candidate should register by directly going to <http://hple.moh.gov.et>. There is no need for re-exam candidates to upload their documents.

Both new and repeat candidates:



- After completing the registration, the candidate must download and print the admission paper by logging into his/her account using username and password, or national ID FAN.
- The candidate can change the exam center by logging into his/her account only during the registration period
- An application for center change is not allowed after the registration period is closed.

2.2. Task Analysis

The first step of exam development involves conducting a comprehensive task analysis study, which identifies the tasks, knowledge, skills, and abilities required from a junior undergraduate professional in the specific profession. The analysis is typically done through surveys, interviews, or observations of practitioners in the actual work environment, as well as through the Delphi method with subject matter experts.

2.3. Exam Blueprint

Based on the task analysis findings, the exam blueprint is prepared that outlines the content areas to be covered in the examination and the weight or emphasis given to each area. This ensures that the exam reflects the key competencies and knowledge needed for competent practice in that specific profession. Blueprint is the matrix or chart that shows the number and type of test questions represented across the topics in the content area, consistent with the learning outcome and relative weight of the test given to each content area. The blueprint also identifies the percentage weighting of cognitive dimensions as the level of competence tested in each knowledge domain.

Key components of blueprint are:

- Domain
- Sub-domain
- Content
- Task
- Process
- Learning outcome
- Assessment methods
- Assessment tools/instrument (test format)
- Relative emphasis (in percentage)

2.4. Item Development

The items are developed following specific guidelines to ensure clarity, relevance, and fairness. Subject matter experts with experience in the field are selected from HEIs to develop exam questions (items) that align with the exam blueprint. The exam questions focus mainly on “knows how” according to the competency level of the Miller's pyramid. The items are produced in a secure location on designated computers that are free from internet connectivity. The items are scenario-based and constructed with stem, lead-in, and four options/alternatives. All items will have a single-best-answer type of Multiple-Choice Question (MCQ) that addresses the learning outcome defined in each content area. Standard textbooks, updated guidelines, and standards are used as reference materials.

2.5. Item Review

Once developed, the items undergo a rigorous review process by item reviewers. The main purpose of the exam review process is to evaluate content relevance, technical accuracy, clarity, and sensitivity related to culture and religion. More experienced subject matter experts as well as psychometric experts will do the review to ensure the items meet psychometric standards. Subject matter experts shall review the items to confirm that they are accurate, clearly stated, and correctly keyed using the checklist. Psychometric experts shall review the items to ensure that

they are not technically flawed. They also work on editorial review to check grammar, punctuation, and spelling errors. This helps ensure the reliability and validity of the items.

2.6. Standard setting method

The standard setting or cut-off point of the EHPLE is determined using the Modified-Angoff method, which is one of the most widely used and legally defensible standard setting approaches to set a cut-off point for high-stake competency examinations.

The method involves a panel of subject matter experts who evaluate each exam question and then estimate the probability that a minimally competent examinee would answer each test item correctly. The average of the experts' predictions for an exam question becomes its predicted difficulty. The average of the predicted difficulty values across all items on a test is the recommended cut-off point. This point indicates the minimum level of knowledge and skill required to pass.

2.7. Exam Administration

The EHPLE is administered following established protocols and guidelines. Proper exam administration procedures, appropriate security measures, and appropriate consideration for test-takers who need special support will be applied during exam administration at exam centers. The exam is administered in selected HEIs nationally, where candidates can choose based on their convenience at the time of registration. The exam schedule will be posted ahead of time on the MoH website and social media pages. Examinees who have a valid admission paper are eligible to sit for the exam. The mode of exam administration is computer-based testing.

CAUTIONS

-
- Candidates are allowed



- Attend the orientation session in order to sit for the exam
 - Arrive at the exam center on time
 - Bring a legal ID and admission paper
 - Complete the exam within the allotted time frame
-

➤ Candidates are **NOT** allowed



- To bring reference materials, blank paper, or notes into the exam center
- To smoke, eat, or drink in the exam room
- To bring mobile phones, tablets, smart watches, camera devices, eyeglasses, calculators or any type of electronic device into the exam center
- To bring their personal belongings to the exam center
- To bring weapons and sharp materials into the exam center
- To give or receive assistance to or from other candidates during the examination

2.8. Scoring and post exam analysis

Once the exam is completed, the scoring process begins. The exam scoring process involves computerized scoring using software.

Post-exam analysis is the process of analyzing examinees' responses to individual test items in order to assess the quality of the items and the exam as a whole. This phase helps to identify any poorly performing items that may need revision or removal from the exam. The item difficulty index, discrimination index, and reliability coefficient are elements of exam analysis.

2.9. Result notification and appeal management

After scoring and analysis, individual score reports are generated and provided to examinees through the website <http://hple.moh.gov.et>. After result notification, examinees can submit their appeal through phone or email within 10 working days after result notification.

2.10. Licensing

The list of examinees who passed the exam will be sent to regional and city administration regulatory bodies. A license is obtained from the regional/zonal health bodies where he/she permanently lives.

Requirements for professional licensing are:



- Passing the EHPLE
- Original or temporary degree
- Educational documents (10th and 12th certificates)
- Medical certificate
- Government issued ID
- Additional prerequisites based on the requirements of regional regulatory bodies

3. Exam Framework

The key broader professional roles, also known as domains or main knowledge areas serve as a building framework for the licensing examination content for Optometry professionals. The domains are further divided into discrete professional attributes that constitute sub-units (also referred to as sub-domains) defining the professional identity of Optometry professionals. Tasks specifying the performance level of each sub- domain serve as the final characteristic of the professional duties on which the licensing exam focuses.

The contents of the licensing examination are presented below, structured into key roles (domains), sub-units (sub-domains), and tasks. The examination emphasis for each domain and sub-domain, out of the total 100% questions, is indicated in brackets.

Key Professional Roles / Domains:

- ☐ Patient Care – **80%**
- ☐ Scholar – **5.5%**
- ☐ Professionalism – **4.0%**
- ☐ Leadership and Management – **4.5%**
- ☐ Health Promotion and Disease Prevention – **6.0%**

Key Role/Domain 1: Patient Care (80%)

Description: The optometry competency framework includes four key areas. Clinical Optometry (57%) is the core domain, focusing on comprehensive eye examinations, diagnosis, and management of visual and ocular conditions. Ophthalmic Dispensing and Glazing (9%) involves interpreting prescriptions, selecting and fitting lenses and frames, and performing basic glazing tasks to ensure optical precision and patient comfort. Optometry Speciality (9%) covers advanced practice areas such as low vision rehabilitation, pediatric optometry, contact lenses, and occupational vision, requiring specialized clinical skills. Surgical Optometry and Operation Theater (5%) includes the optometrist's supportive role in surgical settings, involving pre- and post-operative care and familiarity with theater protocols, ensuring effective collaboration within multidisciplinary teams.

To demonstrate competence in this domain, candidates must apply integrated knowledge in the following sub-units/sub-domains:

- Clinical optometry (**57%**)
- Ophthalmic dispensing & glazing (**9%**)
- Optometry speciality (**9%**)
- Surgical optometry and operation theater (**5%**)

Key Role/Domain 2: Scholar (5.5%)

Description: This domain reflects the role of optometrists as scholars who generate, apply, and share scientific knowledge to enhance eye care services and contribute to the broader healthcare system. It emphasizes the integration of research and evidence-based practice to improve patient outcomes and public eye health. Competency in this domain requires applied knowledge of research methods, biostatistics, epidemiology, health and disease measurement, clinical audit, and research ethics. Optometrists are expected to demonstrate skills in identifying problems, planning and conducting research, collecting and analyzing data, interpreting results, writing reports, and disseminating findings to inform practice and policy.

Key role/ domain 3: Professionalism (4.0%)

Description: This domain encompasses the professional commitment of optometry professionals to promoting the health and well-being of individuals and society through adhering to ethical standards, maintaining personal integrity, and upholding high standards of competence in all areas of practice. To exhibit competence in this domain, candidates must possess applied knowledge of ethical principles, medico-legal practices, effective communication, accountability to the profession and society, maintenance of professional excellence and personal health, continuous professional development and professional values such as, integrity, honesty, altruism, and humility, motivated, compassionate, and respectful care.

Key Role/Domain 4: Leadership and Health System Management (4.5%)

This domain highlights the role of optometrists in contributing to a high-quality healthcare system through self-awareness, leadership, and effective participation in healthcare team development and system management. Optometrists are expected to apply integrated knowledge of continuous quality improvement, health system leadership, management principles, and healthcare ethics. To demonstrate competence in this domain, candidates must possess practical skills to plan, organize, staff, lead, implement, monitor, and manage healthcare resources and activities within their scope of practice.

Key role/ domain 5: Health promotion and disease prevention (6.0%)

Description: This domain reflects the role of optometrists in promoting the eye health and overall well-being of individuals, communities, and populations through advocacy, prevention, and education. It involves actively engaging in disease prevention, health promotion, vision protection, and reducing health disparities. Delivering this service requires an integrated understanding of the social determinants of health, health informatics, epidemiology, communicable and non-communicable eye diseases, and health education strategies aimed at improving public awareness and encouraging healthy visual behaviors.

Table 1 Exam Content for Optometry Profession

Domain 1: Patient Care	
Subdomain 1.1: Clinical optometry	
Content	Task
Clinical refraction	Take history of patient and perform physical examination
	Perform objective refraction
	Perform subjective refraction
	Diagnose refractive and accommodative anomalies
	Manage refractive and accommodative anomalies
Medical optometry	Assess ocular adnexa, anterior segment, posterior segment and neuro-ophthalmology
	Diagnose ocular adnexa and anterior segment eye disease
	Diagnose posterior segment, neuro-ophthalmic diseases and systemic diseases related to ocular disorders
	Diagnose ocular injuries
	Manage ocular adnexa and anterior segment eye disease
	Manage posterior segment and neuro-ophthalmic diseases
	Manage ocular injuries
	Assess ocular adnexa, anterior segment, posterior segment and neuro-ophthalmology
Ophthalmic diagnostic procedure and instrumentations	Operate ophthalmic instruments and interpret results
Sub Domain 1.2: Ophthalmic dispensing and glaze	
Content	Task
Ophthalmic dispensing & glazing	Perform dispensing, glazing and verification of spectacle
Sub domain 1.3: Optometry specialty	
Contents	Tasks
Pediatric optometry	Take pediatric ophthalmic history and perform physical examinations
	Diagnose childhood eye disorder
	Manage childhood eye disorders
Binocular vision, anomalies and vision therapy	Assess physical examination for binocular vision disorder
	Diagnose binocular vision disorders
	Manage binocular vision disorders
Binocular vision, anomalies and vision therapy	Assess physical examination for binocular vision disorders
	Diagnose binocular vision disorders
and vision therapy	Manage binocular vision disorders
Low vision and rehabilitation	Take patient history and perform physical examination

	Manage low vision patients including rehabilitation
Contact lens	Take appropriate history and perform contact lens fitting assessments
	Select appropriate contact lens and manage contact lens complications
Sub Domain 1.4: -Surgical optometry and operation	
Content	Tasks
Surgical optometry	Prepare patients and scrub for surgery
	Perform minor surgeries and provide postoperative care
Domain 2: Leadership and Management	
Tasks	
Lead and manage health care organization, programs and projects	
Domain 3: Health promotion and disease prevention	
Tasks	
Provide community eye care services	
Domain 4: Professionalism	
Tasks	
Adhere to legal, professional and ethical principles	
Domain 5: Scholar	
Tasks	
Conduct descriptive research	

Sample questions

1. A 35-year-old patient has come to an ophthalmic clinic with a complaint of blurring of vision after reading for two hours. On examination, both distance and near visual acuities are normal. The optometrist ordered accommodation fatigue test to rule out the cause of vision reduction at near. What important history has to be taken regarding the patient's symptom?
 - A. Manner of onset
 - B. Diurnal variation
 - C. Precipitating factors
 - D. Laterality of the problem

Answer key: The Answer is **C**

Explanation: Since the patient experiences blurred vision after prolonged near work, it is essential to explore what triggers or worsens the symptom—in this case, activities that demand sustained accommodation, such as reading. Asking about precipitating factors helps the clinician confirm whether the symptom is related to accommodative fatigue or another functional issue, guiding appropriate diagnosis and management.

2. A 13-year-old patient has come to an eye clinic for refractive error check-up. After ascertaining the ocular anomalies, an optometrist planned to perform static retinoscopy. During retinoscopy, the patient is seeing at different direction of gazes. What is the most appropriate retinoscopy preparation in this case?
 - A. Prepare semi-darkened room
 - B. Avoid working distance lens
 - C. Give distance attention holding target
 - D. Being off-centre from patient visual axis

Answer key: The Answer is **C**

Explanation: During static retinoscopy, it is important that the patient maintains steady fixation at a distant target to relax accommodation and allow accurate measurement of refractive error. In this case, the 13-year-old patient is looking in various directions, which can compromise the reliability of the results. Providing a distance fixation target helps the patient maintain consistent gaze and focus, ensuring accurate retinoscopy findings.

3. A 39-year-old patient came to an eye clinic with a complaint of reduction of distance vision since the last two years. During physical examination, the vision of the patient became worse with pinhole. Slit lamp examination also showed central corneal opacity. After physical examination, the optometrist planned to perform static retinoscopy to see retinoscopic reflex. What is the most appropriate retinoscopy technique to be applied in this case?
 - A. Using working distance lens
 - B. Increasing working distance
 - C. Adding high power lens
 - D. Being Off-axis

Answer key: The Answer is **D**

Explanation: In the presence of a central corneal opacity, viewing through the central visual axis may not allow a clear retinoscopic reflex. To overcome this, the examiner can perform retinoscopy from an off-axis position, allowing light to pass through clearer peripheral areas of the cornea. This technique improves visibility of the reflex and enables a more accurate estimation of the refractive error despite the opacity.

4. A 62-year-old patient came to an eye clinic with a complaint of reduction of distance vision on the left eye. During retinoscopy, the optometrist found that a refractive power of OS is $-5.00/-1.00 \times 180$ with VA is CF at 5M. At the end of subjective sphere check, the optometrist found a refractive power of OS is $-4.00/-1.00 \times 180$ with VA is 6/60.

What is the most appropriate JCC power to start cyl-check in this case?

- A. $\pm 1.00D$
- B. $\pm 0.75D$
- C. $\pm 0.50D$
- D. $\pm 0.25D$

Answer key: The Answer is A

Explanation: In patients with **very poor visual acuity**, such as 6/60, it is recommended to start Jackson Cross Cylinder (JCC) testing with a **higher power**, like $\pm 1.00D$, to ensure the patient can detect noticeable differences during the cylindrical refinement. Lower power JCC lenses may not produce a discernible change in blur or clarity for patients with such reduced vision, leading to unreliable responses. Starting with $\pm 1.00D$ allows for more effective detection and refinement of astigmatic correction.

5. A 12-year-old boy came to an eye clinic with a complaint of loss of vision in the left eye for two years. On assessment, the patient has history of surgery following ocular injury on the left eye three years back. After detailed examination, the patient is diagnosed with aphakia. With $+10.00 DS$, the visual acuity improved from CF at 2m to 6/18.

What is the best management option for the patient in this case?

- A. Spectacle correction
- B. Prescribing contact lens
- C. Intraocular lens implantation
- D. Refractive surgery

Answer key: The Answer is C

Explanation: In a 12-year-old patient with aphakia following ocular injury and significant improvement in vision with high plus lenses, the most appropriate long-term management is IOL implantation. This option provides better visual quality, minimizes image size disparity between eyes, and supports binocular vision more effectively than spectacles or contact lenses. Unlike contact lenses, which require daily maintenance, an IOL offers a permanent solution and is more suitable for a child of this age. Refractive surgery is not appropriate due to ongoing ocular development.

6. A man, who uses bifocal eyewear which was prescribed from a hospital for the last one month, came with a complaint of difficulty of reading with his eye glass unless he pushes the eyeglass upward when reading to see through the bifocal.

What is the most appropriate adjustment to correct this problem?

- A. Decrease the vertex distance
- B. Increase the pantoscopic tilt
- C. Decrease the temple bend
- D. Bring the nose pads together

Answer key: The Answer is **D**

Explanation: The patient's complaint — difficulty reading unless he pushes the eyeglass upward — suggests that the bifocal segment is too low relative to his line of sight during near tasks. This is a common fitting issue with bifocals. Bringing the nose pads together raises the frame height on the face, which in turn elevates the bifocal segment, allowing the patient to see through it properly without pushing the glasses up.

7. A five-year-old male child presented to an eye clinic with a penetrating trauma to his right eye. After complete ocular examination, he is diagnosed with corneal tear. An ophthalmologist has scheduled to undertake a corneal tear repair under general anaesthesia. Before surgery, the anaesthetist suggested that the child should fast for eight hours.

What is the most likely reason to make the child fast in this case?

- A. To prevent seizure
- B. To prevent airway closure
- C. To prevent faecal incontinence
- D. To prevent urine incontinence

Answer key: The Answer is **B**

Explanation: While the term "airway closure" isn't textbook phrasing, it likely refers to airway obstruction or compromise due to aspiration. Fasting reduces the risk of aspiration, which can cause airway obstruction and respiratory distress.

8. A patient, who has been under topical corticosteroids for severe uveitis, requested not to use the medication because of perceived difficulty in hearing following medication use. The health professional accepted the complaint and agreed to stop prescribing the medication knowing the possible hearing side effect of the medications.

What is the most likely ethical principle applied in this scenario?

- A. Justice
- B. Autonomy
- C. Beneficence
- D. Nonmaleficence

Answer key: The Answer is **C**

Explanation: The patient's complaint — difficulty reading unless he pushes the eyeglass upward — suggests that the bifocal segment is too low relative to his line of sight during near tasks. This is a common fitting issue with bifocals. Bringing the nose pads together raises the frame height on the face, which in turn elevates the bifocal segment, allowing the patient to see through it properly without pushing the glasses up.

9. An elective cataract surgery was scheduled and informed consent was taken after appropriate discussion with the patient. Unfortunately, formaldehyde was injected instead of lidocaine for anesthetizing eye during the cataract surgery. The professionals were accused in front of the law for the malpractice.

What is the most likely ethical principle violated that leads to lawsuit in this case?

- A. Paternalism
- B. Autonomy
- C. Beneficence
- D. Nonmaleficence

Answer key: The Answer is **D**

Explanation: Nonmaleficence is the ethical principle that means "do no harm." It obligates healthcare professionals to avoid causing injury or harm to patients, whether through acts of commission or omission. In this case, formaldehyde was mistakenly injected instead of lidocaine, which likely caused serious harm to the patient. This constitutes a clear violation of nonmaleficence, as the healthcare providers failed to prevent harm during a routine procedure.

10. An optometrist is taken as role model in the hospital for his patient handling and compassionate care. He talks openly for his patients and he is able to listen and recognize them in appropriate way. However, he is emotionally stable and less reactive to the feeling of patients.

What is the most likely compassionate character that is NOT exhibited from this optometrist?

- A. Empathetic
- B. Motivation
- C. Sensitive
- D. Sympathetic

Answer key: The Answer is **C**

Explanation: In this scenario, the optometrist is described as a role model for compassionate care—he communicates well, listens actively, and is emotionally stable. However, he is noted to be "less reactive to the feelings of patients", which suggests a lack of emotional attunement or responsiveness to patients' emotional states. Sensitivity refers to being emotionally attuned and responsive to others' emotional needs and reactions. The phrase —less reactive to the feeling of patients directly points to a lack of sensitivity. He may be composed and professional, but not emotionally responsive—hence, sensitivity is the missing trait.

11. An eye care team provides a comprehensive eye care service in a rural district. After an ophthalmic investigation, it was found that the community had a 10 percent prevalence of trachomatous inflammation among school age children.

What is the most appropriate way of management to be employed in this case?

- A. Mass topical treatment
- B. Family systemic treatment
- C. Individual topical treatment
- D. Individual systemic antibiotic

Answer key: The Answer is A

Explanation: A 10% prevalence of trachomatous inflammation (TF) among school-age children indicates a public health problem, and according to the World Health Organization (WHO) guidelines, when the prevalence of TF in children aged 1–9 years is $\geq 10\%$, mass antibiotic treatment should be provided to the entire community. However, the correct mass treatment in this context is mass antibiotic treatment, usually with oral azithromycin, not topical. So while "mass treatment" is the correct approach, the correct mode would be systemic, not topical.

12. A researcher conducted a study to determine the prevalence of trachoma and its association with sharing towel among pregnant women who had antenatal care follow-up at public hospitals at the same point in time.

What is the most appropriate study design to be used in this scenario?

- A. Cohort
- B. Case-control
- C. Experimental
- D. Cross-sectional

Answer key: The Answer is D

Explanation: The study measures both the prevalence of trachoma and its association with sharing towels at the same point in time. Cross-sectional studies are observational studies that analyze data from a population at a single point in time. They are suitable for determining the prevalence of a condition and can explore associations between exposure (sharing towels) and outcome (trachoma) simultaneously.

13. A project manager working on quality care for glaucoma patients has been tracking on the progress of each planned activity with the cost incurred and timeline in carrying out each activity. The manager observed that there is an over budget utilization for one of the activities in the plan. The manager also checked that there are two activities that has not yet started, which should be accomplished before the end of the current quarter.

What is the most likely project management phase the manager performs?

- A. Project plan
- B. Project goal
- C. Project evaluation
- D. Project monitoring

Answer key: The Answer is D

Explanation: In this scenario, the project manager is actively tracking the progress of planned activities, comparing actual costs against the budget, and checking timelines to identify any deviations or delays. These tasks are characteristic of the monitoring phase of project management, where the primary focus is on overseeing implementation to ensure the project stays on track. The manager's identification of over-budget utilization and pending activities that risk missing the timeline indicates an effort to assess performance against the project plan. This phase involves continuous tracking, reviewing, and adjusting to maintain alignment with the project's objectives.

14. A newly established general hospital is going to organize staff members for each unit. The hospital developed a job description, specification, and qualifications needed. The hospital posted internal and external vacancies on newspaper and received a pool of applicants.

What is the most appropriate next step of human resource planning that should be applied in this case?

- A. Recruitment
- B. Selection
- C. Job analysis
- D. Retention

Answer key: The Answer is **B**

Explanation: In this scenario, the hospital has already completed job analysis and posted job openings, resulting in a pool of applicants. The next appropriate step is **selection**, which involves reviewing applications, conducting interviews or assessments, and choosing the most suitable candidates from the applicant pool. Selection follows recruitment in the human resource planning process and ensures that the right individuals are hired to meet organizational needs.

15. An optometry staff accused a supervisor for denying a sick leave. The manager called the supervisor to explore the conflict. The supervisor stated that the reason for denying the sick leave was due to repeated request for more than three times within a month. After hearing both sides, the manager was less concerned for the conflict and left the conflict unsolved.

What is the most likely conflict management strategy the manager applied?

- A. Avoidance
- B. Smoothing
- C. Compromise
- D. Collaboration

Answer key: The Answer is **A**

Explanation: In this case, the manager listened to both parties but chose not to take any action to resolve the issue, showing a lack of engagement with the conflict. This approach reflects **avoidance**, a conflict management strategy where the manager sidesteps the issue, hoping it will resolve itself or diminish over time. It often results in unresolved tensions and can affect team morale and performance if used frequently.

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